[redacted]

[redacted]

PROGRESS SHEET

Patient's Name ..... PIMS No .....

Date & Time	
16/8/26 6pm	C/S/B: [redacted] (SR)
	Child reviewed,
	- Last fever spike - 100.9 F @ 12 pm
	- No new complaints
	O/E: Active, playful
	Vitals: stable
	S/E: P/A: soft, NT, liver palpable 1 cm ↓ RCM
	LS - 10.5 cm
	Other S/E: NAD
	plan
	1) To secure IV line
	2) Send CBC, LFTs, CRP
	3) Continue ceftriaxone
	4) W/E complications
17/8/26 7:00 AM	C/S/B - [redacted]
	Δ = ENTERIC FEVER
	C/ Cough ⊕
	No C/o fever spike
	Last fever spike - yesterday at 12 pm - 100.9 °F
	No C/o Nasal discharge
	No C/o Abdominal pain
	No C/o Altered sensorium
	No C/o Vomiting
	No C/o Head Ache

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name .....PIMS No .....

Date & Time	
	O/E: Active, Alert, Afebrile PR – 108/min RR – 22/min SpO2 – 98% @ Room Air
16/3/26 Hb – 9.0 PCV – 28.1 RC – 3.66 Plat – 495 Diff. – 9.72 Count CRP – 8	S/E:- P/A – Soft, non tender, Liver palpable 4 ↓ 2cm Liver span: 9.5cm RS – B/L AE (+) CNS – No focal neurological deficit CVS – S1S2 (+) Plan:- 1) Continue Ceftriaxone 2) W/F Complications
17/3/26 12:00PM	C/S/B – [redacted] Child Reviewed; C/O Cough (+) No c/o fever spike Last fever spike – yesterday at 12pm – 100.7°F No new Complaints O/E: child is Active, Alert, Afebrile

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name .....PIMS No .....

Date & Time	
	PR 102/min RR - 22/min SpO2 - 98% @ Room Air GIE :- P/A - Soft, non tender, Liver is palpable 4 ↓ RCM Liver span - 10.5cm RS : BIL AE (+) CNS : No focal neurological deficit CVS : S1S2 (+) Plan :- 1) If no fever spike change to oral antibiotics and discharge tomorrow

[redacted]

[redacted]